VUGA NDAKUMVA INITIATIVE

CHAPTER 05

Understanding Depression and Depressive Disorders

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Publication status: Evidence-informed educational draft for professional review. This document provides education, not diagnosis, individual treatment, or emergency response. Rwanda-specific pathways must be verified before public release.

Who this chapter serves: General readers, families, community educators and people wanting to understand depression. Content about minors, pregnancy, complex illness or immediate danger requires more specialized support.

Learning objectives

- Explain the difference between ordinary sadness, grief and a depressive disorder.
- Recognize emotional, cognitive, physical, behavioral and safety-related warning signs.
- Understand major depressive disorder and other relevant depressive presentations without self-diagnosing.
- Identify evidence-based professional treatments and the supportive role of self-care.
- Practice low-pressure strategies that support daily functioning and help-seeking.
- Respond compassionately and safely when someone may be depressed or at risk of suicide.

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01 | What is depression?

Depression is a mental health condition that may significantly change mood, interests, energy, thinking and daily functioning. It is more than a brief period of disappointment or feeling unhappy. Some people describe deep sadness; others describe numbness, emptiness, irritability or a loss of interest in things that previously mattered. A person may still go to school or work while experiencing considerable distress.

A depressive episode usually includes persistent depressed mood or markedly reduced interest or pleasure, with other symptoms, lasting for most of the day on nearly every day for at least two weeks. Duration alone does not establish a diagnosis. A qualified clinician also considers intensity, functional impact, personal history, physical conditions and other explanations [1-3].

Depression can affect anyone. It is not a sign of weak character, insufficient faith, laziness or a failure to be grateful. Effective care is available, although people may need different forms and durations of support.

02 | Sadness, grief and clinical depression

Sadness is a normal response to distressing experiences. It can fluctuate during the day and may lessen with comfort, changed circumstances or time. Grief after losing someone important can involve powerful sadness, longing, sleep changes and difficulty concentrating. These experiences are not automatically disorders.

Depression may arise with or without an obvious triggering event and may include loss of pleasure, hopelessness, fatigue and impaired functioning over time. Grief and depression can also coexist. A health professional considers the full context rather than labeling normal human emotion as illness or dismissing symptoms because a loss occurred [1-3].

> Example: Someone feels sad after failing an examination and regains interest in their activities after support and rest. Another person experiences persistent hopelessness, cannot enjoy previous activities and struggles with everyday responsibilities for several weeks. Both deserve compassion; the second pattern particularly merits professional assessment.

03 | Recognizing the signs and symptoms

Depression affects people differently. A person does not need to display every symptom, and symptoms may vary across the lifespan. These signs may also result from physical illness, medication, sleep deprivation or other mental health conditions [1,2].

Emotional and cognitive experiences

- Sad, empty, tearful, low or irritable mood; hopelessness or despair.
- Loss of interest or pleasure, including in relationships and familiar activities.
- Unusually strong guilt, self-criticism, worthlessness or pessimism.
- Difficulty focusing, remembering, deciding or completing tasks.
- Thoughts of death, self-harm or suicide: these need direct, serious attention.

Physical and behavioral experiences

- Changes in sleep, appetite, weight or energy.
- Slowed movements or speech, or observable agitation and restlessness.
- Unexplained or worsening physical complaints that need medical assessment.
- Withdrawing from people, missing responsibilities or reducing personal care.
- Increased use of alcohol or other substances to cope.

04 | How depression changes everyday life

Depression can reduce concentration in class, make household duties feel unusually demanding, strain relationships, interfere with work and reduce enjoyment of community activities. Such changes do not mean the person is unwilling to try. Reduced motivation and energy may be part of the condition.

Severity is not determined only by whether someone can keep working. Someone who appears productive may still have serious symptoms. A clinician looks at distress, safety, duration and functioning rather than making assumptions from appearances [2,3].

05 | Types and patterns of depressive illness

Major depressive disorder: Episodes of depressive symptoms that meet clinical criteria, with a substantial effect on wellbeing or functioning. Episodes may occur once or recur.

Persistent depressive disorder: A longer-lasting pattern of depressed mood that is assessed using specific diagnostic criteria. It may be less dramatic than an acute episode but still seriously affect life.

Depression with seasonal pattern: In some people, depressive episodes follow a recurring seasonal pattern. Ordinary changes in mood during rainy or dry seasons do not establish this diagnosis.

Perinatal depression: Depressive symptoms during pregnancy or after childbirth require assessment and support. This is not the same as temporary, mild mood changes soon after birth.

Depression with psychotic features: Severe depression may sometimes include experiences such as delusions or hallucinations. This requires specialist assessment.

Bipolar depression: Depressive episodes can occur as part of bipolar disorder, which also involves episodes of mania or hypomania. Identifying a history of unusually elevated or irritable mood, reduced need for sleep, and markedly increased activity matters because treatment decisions differ [2,3].

> Important: These descriptions are educational, not a diagnostic checklist. If someone has severely altered perceptions, extreme agitation, inability to care for themselves or immediate danger, urgent professional assessment is appropriate.

06 | Why depression develops

Depression commonly results from interacting biological, psychological and social influences rather than one cause. Family history and other vulnerabilities may matter, but neither genes nor a single difficult event determines a person's future [1,2].

Possible contributing factors include prolonged stress, bereavement, chronic illness or pain, isolation, financial insecurity, discrimination, adverse childhood experiences, trauma, sleep disturbances, changes after childbirth and substance use. Depression can also arise without a clear external explanation.

Practical challenges require practical solutions alongside mental health support. A person who lacks food, housing, safety or income should not be told that positive thoughts alone can resolve their circumstances.

07 | How professionals assess depression

A healthcare professional may ask about mood, pleasure, sleep, appetite, thinking, duration, daily functioning, previous episodes, medications, substance use, physical illness and safety. They may ask directly about suicidal thoughts and any history of unusually high or energized moods. An assessment may include a physical examination or tests when medically appropriate [2,3].

Brief questionnaires can help identify symptoms or monitor changes, but they do not independently confirm a diagnosis. People should have the opportunity to describe their concerns privately and in a language they understand.

08 | Depression at different life stages

Children and adolescents: Depression may appear as irritability, withdrawal, reduced school participation or changes in appetite and sleep. Parents and educators should not dismiss these as misbehavior. Assessment and treatment should be age-appropriate.

Pregnancy and postpartum: Mood changes can have significant consequences for the parent and child. Support should be compassionate and coordinated with maternal healthcare. Treatment decisions require attention to pregnancy or breastfeeding.

Older adults: Depression may coexist with chronic diseases, pain, loneliness or changes in functioning. It must not be dismissed as an inevitable part of aging.

People with chronic illness or disability: Mental and physical conditions may influence each other. Accessible, respectful care that considers both is important [1-3].

09 | Effective treatments for depression

Depression is treatable. The appropriate treatment depends on the severity and course of symptoms, personal preferences, access to services, previous responses and other health needs. Care should be collaborative and reviewed over time [1,3].

Psychological therapies

Cognitive behavioral therapy (CBT) helps people recognize and work with thinking and behavior patterns associated with depression. Behavioral activation supports gradual re-engagement in meaningful and manageable activities. Interpersonal psychotherapy focuses on relationships and life changes. Problem-solving approaches help address practical difficulties in a structured way. These are delivered in formats matched to the person and their needs [1,3].

Medication and combined care

Antidepressants can be appropriate, particularly for more severe depression or when clinical circumstances indicate them. Psychological treatment is often central, and combining therapy with medication may be considered. People should discuss benefits, possible side effects and their preferences with a qualified prescriber. This handbook does not recommend a specific medicine or dose [1,3].

More specialized care

A person with severe, recurrent, persistent, psychotic or treatment-resistant depression may need specialist assessment and additional options. Urgent assessment is required for immediate safety concerns, severe confusion or loss of contact with reality.

10 | Understanding antidepressant treatment

Antidepressants are prescription medicines and do not act as instant mood boosters. Some effects may take time, and a clinician should monitor symptoms, side effects and safety. Response varies, and treatment may need review if it is not helping.

Do not start, stop, change doses or share antidepressants based on information from an article or chatbot. Abruptly stopping medication may cause withdrawal symptoms or a return of symptoms; discuss any planned changes with the prescriber. If agitation or suicidal thoughts emerge or worsen after a medication change, seek prompt clinical advice [3].

11 | Self-care as a support, not a replacement

Self-care can make daily life more manageable, but is not a substitute for medical care when a depressive disorder is present. Because depression can impair energy and concentration, supportive plans should be small, flexible and free from blame.

Examples include maintaining regular waking times where possible, eating and drinking regularly, gentle activity suited to health and ability, spending brief periods outdoors if safe, reducing alcohol, and making contact with a trusted person. Avoid judging someone who cannot manage all these things; financial and health constraints may limit choices.

12 | Practical exercise: small-step activity planning

When someone feels depressed, activities may become harder and withdrawal can reduce opportunities for pleasure or accomplishment. Behavioral activation is a structured treatment approach; the simple exercise below introduces a related, low-pressure self-help skill without replacing therapy [1,3].

- Step 1: Choose one gentle, realistic activity for today - for example washing a cup, sitting outside for five minutes, or greeting a trusted person.
- Step 2: Make the first step very small. If the activity feels overwhelming, reduce it rather than blaming yourself.
- Step 3: Notice what happened before and after the activity. Improvement may be subtle or absent at first.
- Step 4: Consider repeating or adjusting the activity tomorrow. Avoid treating the exercise as a test of success.

- Step 5: If symptoms are severe, persistent or unsafe, prioritize professional assistance rather than increasing self-imposed tasks.

Reflection: One activity I could try is _____. The smallest first step is _____. Someone who might support me is _____. It is fine to leave this blank.

13 | Working with self-critical thoughts

Depression can be accompanied by harsh conclusions such as "I am a burden" or "Nothing will ever improve." Thoughts may feel convincing, but feelings of certainty are not proof. It can help to notice the thought, ask what evidence supports or challenges it, and consider a gentler, more balanced response.

For example, instead of "I failed one task, therefore I am a failure," someone might try "I struggled with this task while feeling unwell; that does not define my worth." This is not about denying real difficulty or forcing positivity.

If reflection becomes repetitive, overwhelming, or increases distress, stop and seek a different form of support. Structured CBT is best taught with a suitably trained practitioner.

14 | Relationships, stigma and asking for help

Depression can make social contact difficult. A person may stop replying to messages, withdraw from friends, or feel ashamed of needing help. Supportive contact can be helpful, provided it respects privacy and individual choice.

A practical request can be specific: "Could we talk for ten minutes?" or "Could you help me arrange a clinic visit?" Sharing only what feels safe is acceptable. Faith, cultural practices and community connections may be sources of comfort, but no one should be pressured to substitute them for needed healthcare.

Stigma creates additional barriers. Statements that depression is laziness, attention-seeking or moral failure can discourage help-seeking and deepen isolation.

15 | Recovery, setbacks and relapse prevention

Recovery does not necessarily happen in a straight line. Some people respond quickly to care; others need more time, ongoing treatment or changes to their plan. Improvement may first appear as better sleep, slightly more energy, less withdrawal or renewed interest in one activity.

With a clinician, people who experience recurrent depression may identify early warning signs, supportive routines, treatment follow-up, safe contacts, and what to do if symptoms return. A setback is a signal to reassess support, not evidence that treatment has failed or that the person has failed [3].

16 | Safety: self-harm and suicidal thoughts

Some people with depression experience thoughts about death, self-harm or suicide, although not everyone with depression does. Such thoughts should always be taken seriously. Direct, calm questions about safety do not need to be avoided; trained professionals assess risk and help establish appropriate care [3].

If someone may be in immediate danger: contact local emergency services or go to the nearest emergency department; seek urgent in-person assistance. If safe to do so, remain with the person until help is available and reduce access to potentially dangerous items without putting yourself at risk. Do not promise secrecy.

If there is no immediate danger but suicidal thoughts are present: encourage prompt assessment by a qualified professional and connection with a trusted, safe support person. Avoid minimizing, arguing, shaming or relying on an AI chatbot for crisis management.

Rwanda-specific emergency numbers and referral pathways must be professionally verified before distribution; this chapter intentionally does not provide an unverified hotline.

> Urgent-care reminder: Sudden confusion, extreme agitation, inability to meet basic needs, psychotic symptoms, or a suicide attempt require urgent medical attention. A digital handbook cannot determine whether someone is safe.

17 | How to support someone who may be depressed

Support begins with listening and practical kindness. Use simple invitations instead of demanding explanations. For example: "I have noticed you seem to be having a difficult time. I care about you. Would you like to talk or would you prefer help finding support?"

Ask permission before sharing personal information. Offer manageable help, such as accompanying someone to a clinic if requested, assisting with daily tasks, or checking in at an agreed time. Avoid statements such as "cheer up," "others have worse problems" or "just try harder."

If someone describes suicidal thoughts, ask directly about immediate safety and help them connect with urgent professional assistance as appropriate. A friend or family member should not be expected to manage serious risk alone.

18 | Realistic learning scenarios

Scenario A: University student

A student who usually enjoys studying begins missing classes, sleeping poorly, avoiding friends and feeling worthless for several weeks. Learning point: This pattern warrants a private conversation and healthcare assessment, not assumptions about laziness. Practical help with accessing care and academic support may matter.

Scenario B: Parent after childbirth

A new parent feels persistently low, detached and overwhelmed and is afraid to admit it. Learning point: Perinatal mental health difficulties deserve confidential, nonjudgmental assessment. The family can offer practical support without replacing professional care.

Scenario C: Worker under financial pressure

A worker loses income and becomes isolated, exhausted and hopeless. Learning point: Mental health care and practical financial or social assistance may both be needed. Reducing the entire problem to attitude or motivation would miss the social context.

Scenario D: Older adult

An older person with chronic pain loses interest in visits and struggles to sleep. Learning point: Physical and mental health should be assessed together. Depression is not simply a normal part of aging.

19 | Seven-day gentle practice guide

This optional plan is not a treatment programme. It is designed to support reflection and small, safe actions; completing all activities is not required. People with significant symptoms should arrange professional assessment instead of relying only on this guide.

Day 1: Notice one change in mood, energy or daily functioning without judging it.

Day 2: Identify one trusted person or professional service you could contact.

Day 3: Choose one very small manageable activity and notice how it goes.

Day 4: If possible, make a simple plan for food, hydration and rest.

Day 5: Write down one harsh self-judgment and a more balanced alternative.

Day 6: Consider one source of practical support for a current difficulty.

Day 7: Review what felt helpful or unhelpful, and decide whether to seek assessment.

20 | Reflection and learning assessment

Personal reflection - optional

- What is one difference between sadness and depression that I understand better now?
- Which change in daily functioning would prompt me to seek professional help?
- What supportive words could I use with someone experiencing depressive symptoms?
- What is one barrier to accessing care in my setting, and what practical support could help?

Knowledge questions

1. Does depression always look like obvious sadness?

2. Can a questionnaire alone diagnose depression?

3. Are therapy and medication the only possible forms of support?

4. Should someone stop prescribed antidepressants abruptly when they feel better?

5. What is the priority when there is immediate danger of suicide?

Answers with explanations

- No. Loss of interest, irritability, numbness, fatigue and changes in functioning may also occur.
- No. Screening tools can assist, but diagnosis requires clinical assessment.
- No. Treatment and recovery can also involve coordinated social, family and practical support; clinical treatment is often crucial.
- No. Medication changes should be discussed with a prescriber because stopping can cause problems.

- Seek emergency in-person assistance, remain with the person if safe, and do not rely on an article or chatbot to assess immediate danger.

21 | Glossary

Depression: A mental health condition involving sustained changes in mood and/or loss of interest alongside other symptoms.

Anhedonia: Reduced ability to experience interest or pleasure.

Depressive episode: A period of depressive symptoms assessed by duration, severity and effect on functioning.

Behavioral activation: An evidence-based approach focused on gradually increasing meaningful and rewarding activity.

CBT: Cognitive behavioral therapy, a structured psychological treatment.

Interpersonal psychotherapy: A treatment that focuses on relationships and important life transitions.

Antidepressant: Prescription medicine used in certain forms of depression and other health conditions.

Relapse: Return of symptoms after a period of improvement.

Safety planning: Collaborative, practical preparation to address personal warning signs and access help during a crisis.

References and source notes

[1] World Health Organization. Depressive disorder (depression). Fact sheet, 11 September 2026. https://www.who.int/news-room/fact-sheets/detail/depression

[2] National Institute of Mental Health. Depression: signs, symptoms, risk factors and treatments. https://www.nimh.nih.gov/health/publications/depression (accessed 9 November 2026).

[3] National Institute for Health and Care Excellence. Depression in adults: treatment and management (NG222). Published 29 June 2022; surveillance review January 2026. https://www.nice.org.uk/guidance/ng222/chapter/Recommendations

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Professional publication checklist

- Clinical review of symptom descriptions, diagnosis, treatment and suicide-safety text.
- Review of adolescent, perinatal, older-adult and bipolar-depression statements by relevant specialists.
- Verification of Rwanda referral routes and local emergency contact information.
- Human-reviewed Kinyarwanda and French translations and accessibility testing.
- Document control: version number, named reviewer, approval date, next review date and correction pathway.